Palbociclib with endocrine therapy

The usual palbociclib starting dose is 125 mg once daily; a lower dose may be prescribed. Palbociclib is taken once daily on Days 1–21, then paused on Days 22–28 of each 28-day cycle. The aromatase inhibitor continues daily, or fulvestrant follows its injection schedule. Ovarian suppression has its own plan when required.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Columns show sequence and general intervals, not equal periods or booked appointments. The team confirms who arranges each test, who reviews results and who authorises treatment or restart.

Check	Before starting	Cycles 1–2	Cycles 3–6	After cycle 6	Hold / change / completion
Treatment	Confirm aromatase inhibitor (AI) or fulvestrant and any ovarian suppression	Palbociclib Days 1–21; rest Days 22–28; fulvestrant loading visits if chosen	Repeat if cleared; endocrine partner continues	Continue while beneficial and tolerated	Confirm each medicine's plan separately
Blood count	Baseline full blood count (FBC)	Day 1 and Day 15 in both cycles	Day 1; extra tests as indicated	Only after at most grade 1–2 neutropenia: may reduce to every three months before a cycle	Low counts, fever or infection require extra review
Renal / liver	Baseline profiles	Before each cycle	Before each cycle	Before each cycle	Review changes before restarting
electrocardiogram (ECG) / echo	Only if clinically indicated	No routine palbociclib-specific ECG date	Investigate symptoms/risk when indicated	Do not copy ribociclib ECG timing	Follow any identified problem
Endocrine / bone / injections	Confirm partner and bone-risk plan	Fulvestrant has its own loading/ injection schedule if used	AI daily or fulvestrant as prescribed	Ovarian suppression and bone review as indicated	Review partner continuation separately
Cancer response	Baseline disease assessment	Review symptoms and tolerability	Imaging according to consultant plan	Continue response and symptom review	Progression prompts a new treatment decision

THE WHOLE TREATMENT JOURNEY

Palbociclib with endocrine therapy: treatment and checks

HR-positive, HER2-negative locally advanced or metastatic breast cancer

The usual palbociclib starting dose is 125 mg once daily; a lower dose may be prescribed. Palbociclib is taken once daily on Days 1–21, then paused on Days 22–28 of each 28-day cycle. The aromatase inhibitor continues daily, or fulvestrant follows its injection schedule. Ovarian suppression has its own plan when required.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Cycles 1–2	Cycles 3–6	After cycle 6	Hold / change / completion
Treatment NCCP 00414 · SMPC	Confirm AI or fulvestrant and any ovarian suppression	Palbociclib Days 1–21; rest Days 22–28; fulvestrant loading visits if chosen	Repeat if cleared; endocrine partner continues	Continue while beneficial and tolerated	Confirm each medicine's plan separately
Blood count NCCP 00414 · SMPC	Baseline FBC	Day 1 and Day 15 in both cycles	Day 1; extra tests as indicated	Only after at most grade 1–2 neutropenia: may reduce to every three months before a cycle	Low counts, fever or infection require extra review
Renal / liver NCCP 00414 · SMPC	Baseline profiles	Before each cycle	Before each cycle	Before each cycle	Review changes before restarting
ECG / echo NCCP 00414 · SMPC	Only if clinically indicated	No routine palbociclib-specific ECG date	Investigate symptoms/risk when indicated	Do not copy ribociclib ECG timing	Follow any identified problem
Endocrine / bone / injections NCCP 00414 · SMPC	Confirm partner and bone-risk plan	Fulvestrant has its own loading/ injection schedule if used	AI daily or fulvestrant as prescribed	Ovarian suppression and bone review as indicated	Review partner continuation separately
Cancer response NCCP 00414 · SMPC	Baseline disease assessment	Review symptoms and tolerability	Imaging according to consultant plan	Continue response and symptom review	Progression prompts a new treatment decision

What each check is for

1 Before the first dose

Before starting

Sources: NCCP 00414 · SMPC

2 Early toxicity checks

Cycles 1–2

Sources: NCCP 00414 · SMPC

3 Benefit, tolerability and adherence

After cycle 6

Sources: NCCP 00414 · SMPC

4 Completion or treatment change

Hold / change / completion

Sources: NCCP 00414

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.21 · Sources checked 2026-10-25

Source reconciled; independent clinical, pharmacy and patient/PPI review pending